

APO-WARFARIN

Warfarin Sodium Tablets USP Anti-Coagulant

COMPOSITION:

Apo-Warfarin 1mg, 2mg and 5mg Tablets

Each tablet contains Warfarin Sodium Tablets USP

PHARMACOLOGICAL ACTIONS:

Warfarin and other coumarin anticoagulants act by inhibiting the synthesis of Vitamin K dependent clotting factors, which include Factors II, VII, IX and X, and the anticoagulant proteins C and S. Half-life of these clotting factors are as follows: Factor II: 60 hours, VII: 4-6 hours, IX: 24 hours and X: 48-72 hours. The half-lives of proteins C and S are approximately 8 hours and 30 hours, respectively. The resultant *in vivo* effect is a sequential depression of Factors VII, IX, X and II. Vitamin K is an essential cofactor for the post ribosomal synthesis of the vitamin K dependent clotting factors. The vitamin promotes the biosynthesis of γ -carboxyglutamic acid residues in the proteins, which are essential for biological activity. Warfarin is thought to interfere with clotting factor synthesis by inhibition of the regeneration of vitamin K1 epoxide. The degree of depression is dependent upon the dosage administered. Therapeutic doses of warfarin decrease the total amount of the active form of each vitamin K dependent clotting factor made by the liver by approximately 30% to 50%. An anticoagulation effect generally occurs within 24 hours after drug administration. However, peak coagulant effect may be delayed 72 to 96 hours. The duration of action of a single dose of racemic warfarin is 2 to 5 days. The effects of warfarin may become more pronounced as effects of daily maintenance doses overlap. Anticoagulants have no direct effect on an established thrombus, nor do they reverse ischemic tissue damage. However, once a thrombus has occurred, the goal of anticoagulant treatment is to prevent further extension of the formed clot and prevent secondary thromboembolic complications, which may result in serious and possibly fatal sequelae. There are no differences in the apparent volume of distribution after intravenous and oral administration of single doses of warfarin solution. Warfarin distributes into a relatively small apparent volume of distribution of about 0.14 litre/kg. A distribution phase lasting 6-12 hours is distinguishable after rapid intravenous or oral administration of an aqueous solution.

INDICATIONS:

APO-WARFARIN (warfarin sodium) is indicated for the prophylaxis and/or treatment of venous thrombosis and its extension, pulmonary embolism, atrial fibrillation with embolization, and as an adjunct in the prophylaxis of systemic embolism after myocardial infarction, including stroke, reinfarction and death. The following are some of the more common clinical disorders which may be associated with or predispose patients to the above indications:

1. Thrombophlebitis
2. Congestive heart failure
3. Surgical procedure or trauma associated with a high risk of thromboembolism.
4. Myocardial infarction
5. Cerebral embolism and treatment of transient cerebral ischemic attacks due to intravascular clotting.

ADVERSE REACTIONS:

Fatal or nonfatal haemorrhage from any tissue or organ. This is a consequence of the anticoagulant effect. The signs, symptoms, and severity will vary according to the location and degree or extent of the bleeding. Haemorrhagic complications may present as paralysis; parathesia; headache, chest, abdomen, joint, muscle or other pain; dizziness; shortness of breath, difficult breathing or swallowing; unexplained swelling; weakness; hypotension; or unexplained shock. Therefore, the possibility of haemorrhage should be considered in evaluating the condition of any anticoagulated patient with complaints which do not indicate an obvious diagnosis. Bleeding during coagulant therapy does not always correlate with PT ratio/INR.

Bleeding which occurs when the PT/INR is within the therapeutic range warrants diagnostics investigation since it may unmask a previously unsuspected lesion, e.g. tumour, ulcer, etc.

Necrosis of skin and other tissues

Body as a whole: Hypersensitivity/allergic reactions, pain, edema, asthenia, fever, headache, fatigue, lethargy, malaise. Central and Peripheral Nervous System: dizziness, cold intolerance and paresthesia, including feeling cool and chills. Gastrointestinal: nausea, diarrhea, abdominal pain, including cramping, flatulence/bloating, vomiting.

Liver and Biliary: elevated liver enzymes, hepatitis, jaundice, cholestatic injury.

Skin and Appendages: alopecia, rash, pruritus, urticaria, dermatitis, including bullous eruptions.

Calciphylaxis: frequency 'not known'.

Vascular, Extracardiac: systemic cholesterol microembolization, purple toes syndrome, vasculitis.

Special Senses: taste perversion.

Rare events of tracheal or tracheobronchial calcification have been reported in association with long-term warfarin therapy. The clinical significance of this event is unknown.

Priapism has been associated with anticoagulant administration; however, a causal relationship has not been established.

Renal and urinary disorders

Frequency 'not known': Anticoagulant-related nephropathy

PRECAUTIONS/WARNINGS:

The most serious risks associated with anticoagulations therapy with sodium warfarin are haemorrhage in any tissue or organ and, less frequently (<0.1%), necrosis and/or gangrene of skin and other tissues. The risk of haemorrhage is related to the level of intensity and the duration of anticoagulant therapy. Haemorrhage and necrosis have, in some cases, been reported to result in death or permanent disability. Necrosis appears to be associated with local thrombosis and usually appears within a few days of the start of anticoagulant therapy. In severe cases of necrosis, treatment through debridement or amputation of the affected tissue, limb, breast or penis has been reported. Careful diagnosis is required to determine whether necrosis is caused by an underlying disease. Warfarin therapy should be discontinued when warfarin is suspected to be the cause of developing necrosis and heparin therapy may be considered for anticoagulation. Although various treatments have been attempted, no treatment for necrosis has been considered uniformly effective. These and other risks associated with anticoagulant therapy must be weighed against the risk of thrombosis or embolization in untreated cases.

Heparin prolongs the one-stage PT. When heparin and Apo- Warfarin are administered concomitantly, refer to

Conversion from Heparin Therapy under Recommended Dosage, Dosage Schedule and Route of Administration.

Systemic atheroemboli and cholesterol microemboli can present with a variety of signs and symptoms including "purple toes syndrome", livedo reticularis, rash gangrene, abrupt an intense pain in the leg, foot or toes, foot ulcers, myalgia, penile gangrene, abdominal pain, flank or black pain, hematuria, renal insufficiency, hypertension, cerebral ischemia, spinal cord infarction, pancreatitis, symptoms simulating polyarteritis, or any other sequelae of vascular compromise due to embolic occlusion. The most commonly involved visceral organs are the kidneys followed by the pancreas, spleen and liver. Some cases have progressed to necrosis or death.

"Purple toes syndrome" is a complication of oral anticoagulation characterized by a dark, purplish or mottled color of the toes, usually occurring between 3-10 weeks, or later, after the initiation of therapy with warfarin or related compounds. Major features of this syndrome includes purple color of plantar surfaces and sides of the toes that blanches on moderate pressure and fades with elevation of the legs; pain and tenderness of the toes; waxing and waning of the color over time. While the "purple toes syndrome" is reported to be reversible, some cases progress to gangrene or necrosis which may require debridement of the affected area, or lead to amputation.

A severe elevation (>50 sec) in activated partial thromboplastin time (PTT) with a PT/INR in the desired range has been identified as an indication of increased risk of postoperative hemorrhage.

The decision to administer anticoagulants in the following conditions must be based upon clinical judgment in which the risks of anticoagulant therapy are weighed against the benefits:

Severe to moderate hepatic or renal insufficiency.

Infectious diseases or disturbances of intestinal flora: Sprue, antibiotic therapy.

Trauma which may result in internal bleeding.

Surgery or trauma resulting in large exposed raw surfaces. Indwelling catheters.

Severe to moderate hypertension.

Known or suspected deficiency in protein C-mediated anti-coagulant response: Hereditary or acquired deficiencies of protein C or its co-factor, protein S, have been associated with tissue necrosis following warfarin administration. Not all patients with these conditions develop necrosis, and tissue necrosis occurs in patients without these deficiencies. Inherited resistance to activated protein C has been described in many patients with venous thromboembolic disorders but has not yet been evaluated as a risk factor for tissue necrosis. The risk associated with these conditions, both for recurrent thrombosis and for adverse reactions, is difficult to evaluate since it does not appear to be the same for everyone. Decisions about testing and therapy must be made on an individual basis. It has been reported that concomitant anti-coagulation therapy with heparin for 5-7 days during initiation of therapy with Apo-Warfarin may minimize the incidence of tissue necrosis. Warfarin therapy should be discontinued when warfarin is suspected to be the cause of developing necrosis and heparin therapy may be considered for anticoagulation.

Periodic determination of PT ratio/INR or other suitable coagulation test is essential.

Medications of unknown interaction with coumarins are best regarded with caution. When these medications are started or stopped, more frequent PT ratio/INR monitoring

is advisable. Coumarins may also affect the action of other drugs. Hypoglycaemic agents (chlorpropamide and tolbutamine) and anticonvulsants (phenytoin and phenobarbital) may accumulate in the body as a result of interference with either their metabolism or excretion.

It has been reported that concomitant administration of warfarin and ticlopidine may be associated with cholestatic hepatitis.

Intramuscular injections of concomitant medications should be confined to the upper extremities, which permits easy access for manual compression, inspections for bleeding and use of pressure bandages.

Close monitoring of patients receiving nonsteroidal anti-inflammatory agents (NSAIDs) is recommended to be certain that no change in anticoagulation dosage is required. In addition to specific drug interactions that might affect prothrombin time, NSAIDs can inhibit platelet aggregation, and can cause gastrointestinal bleeding, peptic ulceration and/or perforation.

Acquired or inherited warfarin resistance should be suspected if large daily doses of APO-WARFARIN are required to maintain a patient's PT ratio/INR within a normal therapeutic range.

Calciophylaxis is a rare syndrome of vascular calcification with Cutaneous necrosis, associated with high mortality. The condition is mainly observed in patients with end-stage renal disease on dialysis or in patients with known risk factors such as protein C or S deficiency, hyperphosphatemia, hypercalcaemia or hypoalbuminaemia. Rare cases of calciophylaxis have been reported in patients taking warfarin, also in the absence of renal disease. In case calciophylaxis is diagnosed, appropriate treatment should be started and consideration should be given to stopping treatment with warfarin.

Topical preparations containing methyl salicylate should be used with care in patients on Warfarin and excessive usage is to be avoided as potentially dangerous drug interaction can occur.

Co-administration with topical miconazole (see Drug Interactions).

Anticoagulant-related nephropathy

In patients with altered glomerular integrity or with a history of kidney disease, acute kidney injury may occur, possibly in relation to episodes of excessive anticoagulation and hematuria. A few cases have been reported in patients with no pre-existing kidney disease. Close monitoring including a renal function evaluation is advised in patients with a supratherapeutic INR and hematuria (including microscopic).

Use in Elderly and/or Debilitated Patients: Patients 60 years or older appear to exhibit greater than expected PT/INR response to the anticoagulant effects of warfarin. Caution should be exercised with administration of warfarin sodium to elderly and/or debilitated patients in any situation or physical condition where added risk of haemorrhage is present.

Use in Children: Safety and effectiveness in children below 18 years of age have not been established in randomized, controlled clinical trials. However, the use of warfarin in pediatric patients has been documented for the prevention and treatment of thromboembolic events. Difficulty achieving and maintaining therapeutic PT ratio/INR ranges in the pediatric patient has been reported. More frequent PT ratio/INR determinations are recommended because of possible changing warfarin requirements.

Topical preparations containing methyl salicylate should be used with care in patients on Warfarin and excessive usage is to be avoided as potentially dangerous drug interaction can occur.

CONTRAINDICATIONS:

Anticoagulation is contraindicated in any localized or general physical condition or personal circumstances in which the

hazard of haemorrhage might be greater than the potential clinical benefits of anticoagulation such as:

Pregnancy: Apo-Warfarin (warfarin sodium) is contraindicated in pregnancy because the drug passes through the placental barrier and may cause fatal haemorrhage to the fetus *in utero*. Women of childbearing potential must take precautions not to become pregnant while on APO-WARFARIN therapy.

Furthermore, there have been reports of birth malformations in children born to mothers who have been treated with warfarin during pregnancy.

Embryopathy characterized by nasal hypoplasia with or without stippled epiphyses (chondrodysplasia punctata) has been reported in pregnant women exposed to warfarin during the first trimester. Central nervous system abnormalities also have been reported, including dorsal midline dysplasia characterized by agenesis of the corpus callosum, Dandy-Walker malformation, and midline cerebellar atrophy. Ventral midline dysplasia, characterized by optic atrophy, and eye abnormalities have been observed. Mental retardation, blindness, and other central nervous system abnormalities have been reported in association with second and third trimester exposure. Although rare, teratogenic reports following *in utero* exposure to warfarin include urinary tract anomalies such as single kidney, asplenia, anencephaly, spina bifida, cranial nerve palsy, hydrocephalus, cardiac defects and congenital heart disease, polydactyly, deformities of toes, diaphragmatic hernia, corneal leukoma, cleft palate, cleft lip, schizencephaly, and microcephaly.

Spontaneous abortion and still birth are known to occur and a higher risk of fetal mortality is associated with the use of warfarin. Low birth weight and growth retardation have also been reported.

Women of childbearing potential who are candidates for anticoagulant therapy should be carefully evaluated and the indications critically reviewed with the patient. If the patient becomes pregnant while taking this drug, she should be apprised for the potential risks to the fetus, and the possibility of termination of the pregnancy should be discussed in the light of those risks.

Haemorrhagic tendencies or blood dyscrasias. Recent or contemplated surgery of:

1. central nervous system
2. eye
3. traumatic surgery resulting in large open surfaces. Bleeding tendencies associated with active ulceration or overt bleeding of:
 1. gastrointestinal, genitourinary or respiratory tracts
 2. cerebrovascular haemorrhage
 3. aneurysms—cerebral, dissecting aorta
 4. pericarditis and pericardial effusions
 5. bacterial endocarditis

Threatened abortion, eclampsia and preeclampsia. Inadequate laboratory facilities.

Unsupervised patients with senility, alcoholism, or psychosis or other lack of patient cooperation.

Spinal puncture and other diagnostic or therapeutic procedures with potential for uncontrollable bleeding.

Miscellaneous: major regional, lumbar block anesthesia, malignant hypertension and known hypersensitivity to warfarin sodium or to any other components of APO- WARFARIN.

Co-administration with miconazole oral gel (see Drug Interactions).

DRUG INTERACTIONS:

Drugs may interact with Apo-Warfarin through pharmacodynamic or pharmacokinetic mechanisms. Pharmacodynamic mechanisms for drug interactions with Apo-Warfarin are synergism (impaired hemostasis, reduced clotting factor synthesis), competitive antagonism (vitamin K), and altered physiologic control loop for vitamin K metabolism (hereditary resistance). Pharmacokinetic mechanisms for drug interactions with Apo-Warfarin are mainly enzyme induction, enzyme inhibition, and reduced plasma protein binding. It is important to note that some drugs may interact by more than one mechanism.

The following drugs may be responsible for INCREASED PT ratio or INR response: Acetaminophen; alcohol*; allopurinol; aminosalicic acid; amiodarone HCl; ASA; azithromycin; capecitabine; cefamandole; cefazolin' cefoperazone; cefotetan; ceftioxin; ceftriaxone; celecoxib; chenodiol; chloramphenicol; chloral hydrate*; chlorpropamide; cholestyramine*; cimetidine; ciprofloxacin; cisapride; clarithromycin; clofibrate; cyclophosphamide*; danazol; danshen; dextran; dextrothyroxine; diazoxide; diclofenac; dicumarol; diflunisal; disulfiram; doxycycline; erythromycin; ethacrynic acid; fenofibrate; fenoprofen; fluconazole; fluorouracil; fluoxetine; flutamide; fluvastatin; fluvoxamine; gemfibrozil; glucagon; halothane; heparin; ibuprofen; ifosfamide; indomethacin; influenza virus vaccine; itraconazole; ketoprofen; ketorolac; levamisole; levothyroxine; liothyronine; lovastatin; mefenamic acid; methimazole*; methyldopa; methylphenidate; methylsalcylate ointment; metronidazole; miconazole; moricizineHCl*; nalidixic acid; naproxen; neomycin; norfloxacin; ofloxacin; olsalazine; omeprazole; oxaprozin; oxymetholone; paroxetine; penicillin G, IV; pentoxifylline; phenylbutazone; phenytoin*; piperacillin; piroxicam; prednisolone*; propafenone; propoxyphene; propranolol; propylthiouracil*; quinidine; quinine; ranitidine*; rofecoxib; sertraline; simvastatin; stanozolol; streptokinase; sulfamethizole; sulfamethoxazole; sulfinpyrazone; sulfisoxazole; sulindac; tamoxifen; tetracycline; thyroid; ticarcillin; ticlopidine; t-PA; tolbutamide; trimetoprim/sulfamethaxazole; urokinase; valproate; vitamin E; warfarin overdose; zafirlukast.

The following drugs may be responsible for DECREASED PT ratio or INR response:

Alcohol*; aminogluthethimide; amobarbital; atorvastatin; azathioprine; butabarbital; butalbital; carbamazepine; chloral hydrate*; chlordiazepoxide; chlorthalidone; cholestyramine*; corticotropin; cortisone; cyclophosphamide*; dicloxacillin; ethchlorvynol; glutethimide; griseofulvin; haloperidol; meprobamate; 6- mercaptopurine; methimazole*; moricizine HCl*; nafcillin; paraldehyde; pentobarbital; phenobarbital; phenytoin*; prednisolone*; primidone; thiouracil*; ranitidine*; rifampin; secobarbital; St.John's Wort; spironolactone; sucralfate; trazodone; vitamin C(high dose); vitamin K; warfarin underdosage.

* Increased and decreased PT ratio/INR responses have been reported.

RECOMMENDED DOSAGE, DOSAGE SCHEDULE and ROUTE of ADMINISTRATION:

Administration: The administration and dosage of APO- WARFARIN (warfarin sodium) must be individualized according to the patient's responsiveness to the drug. The dosage should be adjusted according to results of the patient's PT ratio/INR. Measurement of warfarin induced

effects on PT can vary substantially due to the sensitivity of different thromboplastin reagents.

Venous Thromboembolism (including pulmonary embolism): An INR of 2–3 is sufficient for prophylaxis and treatment of venous thromboembolism and minimizes the risk of hemorrhage associated with higher INRs.

Atrial Fibrillation: For non-valvular atrial fibrillation, the effects of warfarin in reducing thromboembolic events including stroke were similar at either moderately high INR (2–4.5) or low INR (1.4–3). There was a significant reduction in minor bleeds at the low INR. No data are available for valvular atrial fibrillation patients.

Post-Myocardial Infarction: Apo-Warfarin therapy should be initiated early in 2–4 weeks post-infarction and dosage should be adjusted to maintain an INR of 2.5–3.5 long term based on WARIS study. In patients thought to be at an increased risk of bleeding complications or on aspirin therapy, maintenance of Apo-Warfarin therapy at the lower end of this INR range is recommended.

Mechanical and Bioprosthetic Heart Valves: In patients with mechanical heart valve(s), long-term prophylaxis with warfarin to an INR of 2.5–3.5 is recommended. In patients with bioprosthetic heart valve(s), the American College of Chest Physicians recommends warfarin therapy to an INR of 2–3 for 12 weeks after valve insertion. In patients with additional risk factors, eg atrial fibrillation or prior thromboembolism, consideration should be given for long term therapy.

Recurrent Systemic Embolism: In cases where the risk of thromboembolism is great, eg in patients with recurrent systemic embolism, a higher INR may be required.

An INR of >4 appears to provide no additional therapeutic benefit in most patients and associated with a higher risk of bleeding.

Initial Dosage – The dosing of APO-WARFARIN must be individualized according to the patient's response to the drug as indicated by the INR and/or PT ratio. It is recommended that APO-WARFARIN therapy be initiated with a dose of 2 to 5 mg per day with dosage adjustments based on the results of INR and/or PT ratio determinations. Low initiation doses are recommended for elderly and/or debilitated patients and patients with potential for increased responsiveness to warfarin. Elderly and Asian patients may require lower initiation and maintenance doses of APO-WARFARIN. Use of a large loading dose may increase the incidence of haemorrhagic and other complications, does not offer more rapid protection against thrombi formation, and is not recommended.

Maintenance – Most patients are satisfactorily maintained at a dose of 2 to 10 mg daily. Flexibility of dosage is provided by breaking scored tablets in half. The individual dose and interval should be gauged by the patient's prothrombin response.

Duration of therapy – The duration of therapy in each patient should be individualized. In general, anticoagulant therapy should be continued until the danger of thrombosis and embolism has passed.

Missed Dose – The anticoagulant effect of warfarin persists beyond 24 hours. If the patient forgets to take the prescribed dose of APO-WARFARIN at the scheduled time, the dose should be taken as soon as possible on the same day. The patient should not take the missed dose by doubling the daily dose to make up for missed doses, but should refer back to his or her physician.

LABORATORY CONTROL – The INR reflects the depression of vitamin K dependent Factors VII, X and II. The INR should be

determined daily after the administration of the initial dose until INR results stabilize in the therapeutic range. Intervals between subsequent INR determinations should be based upon the patient's INR response and the physician's judgement of the patient's reliability. For example, INR may be monitored two or three times weekly for one to two weeks, then less often, depending on the stability of the INR results. If the INR response remains stable, the frequency of testing may be reduced with intervals as long as every four to six weeks for appropriate patients.

To ensure adequate control, it is recommended that additional PT tests are done when other warfarin products are interchanged with APO-WARFARIN and also if other medications are coadministered with APO-WARFARIN.

In switching to another warfarin product, particular emphasis needs to be placed on INR control. INR outside of the therapeutic range may result in serious clinical consequences: lack of efficacy leading to thromboembolic stroke or myocardial infarction, if INR values are low, and intracranial bleeding if they are high.

TREATMENT DURING DENTISTRY AND SURGERY – The

management of patients who undergo dental and surgical procedures requires close liaison between attending physicians, surgeons and dentists. PT ratio/INR determination is recommended just prior to any dental or surgical procedure. In patients undergoing minimal invasive procedures who must be anticoagulated prior to, during, or immediately following these procedures, adjusting the dosage of APO-WARFARIN to maintain the PT ratio/INR at the low end of the therapeutic range, may safely allow for continued anticoagulation. The operative site should be sufficiently limited and accessible to permit the effective use of local procedures for haemostasis. Under these conditions, dental and surgical procedures may be performed without undue risk of haemorrhage. Some dental or surgical procedures may necessitate the interruption of APO-WARFARIN therapy. When discontinuing APO-WARFARIN even for a short period of time, the benefits and risks should be strongly considered.

CONVERSION FROM HEPARIN THERAPY – Since the

anticoagulant effect of warfarin is delayed, heparin is preferred initially for rapid anticoagulation. Conversion to APO-WARFARIN may begin concomitantly with heparin therapy or may be delayed 3 to 6 days. To ensure continuous anticoagulation, it is advisable to continue full dose heparin therapy and that APO-WARFARIN therapy be overlapped with heparin for 4 to 5 days, until APO-WARFARIN has produced the desired therapeutic response as determined by PT ratio/INR. When APO-WARFARIN has produced the desired PT ratio/INR or prothrombin activity, heparin may be discontinued.

APO-WARFARIN may increase the aPTT test, even in the absence of heparin. During initial therapy with APO-WARFARIN, the interference with heparin anticoagulation is of minimal clinical significance.

As heparin may affect the PT, patients receiving both heparin and APO-WARFARIN should have blood drawn for PT ratio/INR determination, at least:

5 hours after the last IV bolus dose of heparin, or

4 hours after cessation of a continuous IV infusion of heparin, or

24 hours after last subcutaneous heparin injection.

SYMPTOMS and TREATMENT OF OVERDOSAGE and ANTIDOTE (S):

Signs and Symptoms: Suspected or overt abnormal bleeding (e.g., appearance of blood in stools or urine, haematuria, excessive menstrual bleeding, melena, petechiae, excessive bruising or persistent oozing from superficial injuries) are early manifestations of anticoagulation beyond a safe and satisfactory level.

Treatment: Excessive anticoagulation, with or without bleeding, may be controlled by discontinuing APO- WARFARIN (warfarin sodium) therapy and if necessary, by administration of oral or parenteral vitamin K1 (please see recommendations accompanying vitamin K1 preparations prior to use).

Such use of vitamin K1 reduces responses to subsequent warfarin therapy. Patients may return to a pretreatment thrombotic status following the rapid reversal of a prolonged PT. Resumption of warfarin administration reverses the effect of vitamin K1, and a therapeutic PT can again be obtained by careful dosage adjustment. If rapid anticoagulation is indicated, heparin may be preferable for initial therapy.

If minor bleeding progresses to major bleeding, give 5 to 25 mg (rarely up to 50 mg) parenteral vitamin K1. In emergency situations of severe haemorrhage, clotting factors can be returned to normal by administering 200 to 500 mL of whole blood or fresh frozen plasma, or by giving commercial Factor IX complex.

A risk of hepatitis and other viral diseases is associated with the use of these blood products; Factor IX complex is also associated with an increased risk of thrombosis. Therefore, these preparations should be used only in exceptional or life-threatening bleeding episodes secondary to warfarin overdosage.

Purified Factor IX preparations should not be used because they cannot increase the levels of prothrombin, factor VII and Factor X, which are also depressed along with the levels of Factor IX as a result of warfarin treatment, packed red blood cells may also be given if significant blood loss has occurred. Infusions of blood or plasma should be monitored carefully to avoid precipitating pulmonary edema in elderly patients or patients with heart disease.

PACKING/PACK SIZES:

APO-WARFARIN 1mg, 2mg and 5mg. Packed in 100's in HDPE bottles.

STORAGE CONDITIONS:

Store below 30°C. Protect from light. Keep containers closed.

DESCRIPTION:

Apo-Warfarin 1mg tablet: Pink and round biconvex tablet scored and engraved 'WAR' over 1 on one side, 'APO' other side.

Apo-Warfarin 2mg tablet: Lavender and round biconvex tablet scored and engraved 'WAR' over 2 on one side, 'APO' other side.

Apo-Warfarin 5mg tablet: Peach and round biconvex tablet scored and engraved 'WAR' over 5 on one side, 'APO' other side.

MANUFACTURER: Apotex Inc., 150 Signet Drive, Toronto (Weston), Ontario Canada, M9L 1T9.

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Apotex Inc.

**150 Signet Drive, Weston
Ontario, Canada M9L 1T9**

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